



Guidelines

Acute Anaphylaxis, Management of (Anaphylaxis Pathway)

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Medical and Nursing

Introduction

Anaphylaxis is a severe form of allergic reaction that is potentially life-threatening, especially if not treated immediately. It is characterised by a sudden onset; however, the clinical presentation is variable. The diagnosis of anaphylaxis is based on clinical findings and takes the patient's history and physical examination into consideration.¹

Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) is committed to the delivery of safe and high-quality person-centred care. This guideline aims to provide evidence-based guidance on the management of acute anaphylaxis in accordance with the Australian Commission on Safety and Quality in Health Care (ACSQHC) [Acute Anaphylaxis Clinical Care Standard](#) and the [National Safety and Quality Health Service \(NSQHS\) Standards \(National Standards\) Standard 8: Recognising and Responding to Acute Deterioration](#).

This guideline should be read in conjunction with the [SCGOPHCG Recognising and Responding to Acute Deterioration Procedure](#) and [SCGOPHCG Clinical Alerts Policy](#), SCGOPHCG [NPG Intravenous Therapy](#).

The Anaphylaxis Pathway should be used in ED, the wards, or the community when an acute presentation of anaphylaxis is suspected. Anaphylaxis should be considered in patients with a previously known allergy or anaphylaxis presents with similar symptoms, or when a patient with no previous allergy history develops signs and symptoms consistent with anaphylaxis after exposure to an allergen or new medication (PO, SC, IM, IV, topical, contact).

Risk Statement

Non-compliance with this guideline may lead to:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Definitions

Anaphylaxis

[The Australian Society of Clinical Immunology and Allergy \(ASCI\)](#) defines anaphylaxis as²:

- Any acute onset illness with typical skin features (urticarial rash or erythema/flushing, and/or angioedema), plus involvement of respiratory and/or cardiovascular and/or persistent severe gastrointestinal symptoms; or
- Any acute onset of hypotension or bronchospasm or upper airway obstruction where anaphylaxis is considered possible, even if typical skin features are not present.

Allergy history taking

All patients accessing healthcare across SCGOPHCG onsite and offsite will be assessed for their allergy history including whether they have had an anaphylaxis reaction and if they carry an adrenaline injector (e.g., EpiPen®).

All staff involved in care must be informed that the patient has an allergy with anaphylaxis reaction. This includes at clinical handover in the ward and in the relevant HITH/Home Link community setting or when the patient temporarily leaves the ward, such as attending scans or other tests. The patient should be given a red patient identification band to alert for allergy risks (excluding HITH/Home Link patients), an allergy sticker placed on their medication chart and all other relevant HITH/Home Link documentation such as the admission sheet and medication chart. An allergy alert will be added to the patient's healthcare records in accordance with [SCGOPHCG Clinical Alerts Policy](#). An allergy alert sign (or similar) should be placed at the patient's bedside and on patient journey boards in an applicable ward setting or on the patient's file in the community setting.

Access to personal adrenaline injector

For adrenaline to be given as soon as possible after the onset of symptoms of anaphylaxis, it is important for the patient (or their carer) to be able to immediately administer their own adrenaline injector, regardless of their physical location within SCGOPHCG.¹ A readily accessible patient's own adrenaline injector may also be used by a clinician in emergency situations if deemed necessary (i.e. delayed access to MET equipment). If a patient has an adrenaline injector, as soon as possible following presentation or admission, their capacity to safely use it during their admission at SCGOPHCG will be assessed. This includes:

- Assessing their capacity, physical capability and willingness to use their personal adrenaline injector and their ability to recognise the symptoms of anaphylaxis.
- Considering medicines administered during their admission that may impair the patient's usual ability to recognise anaphylaxis and use an adrenaline injector.
- Involving a family member or carer in the assessment, if appropriate, when a patient is cognitively impaired or lives with a disability.
- Checking the expiry date and integrity of the adrenaline injector. Checking the device to make sure the solution is clear, does not appear to have been used, and has been stored appropriately. If the device is expired or there are any doubts about the integrity of the patient's own adrenaline injector, obtain a replacement device from pharmacy.
- Ensuring that storage of and access to the adrenaline injector does not compromise the safety of others.

After the assessment, the adrenaline injector should be labelled with the patient's name using a pre-printer ID label, and is to stay with the patient at all times during their admission (including transfer i.e. procedures etc. It is to be stored at the bedside or in an area which can't be accessed by other patients (patient's guardians may also administer)

To ensure that patients, guardians and staff are aware of the availability of a Patient Own Medication adrenaline injector, pharmacists will note such storage in the Medication History and Management Plan (MHMP) MR 410/804.

All staff involved in care must be informed that the patient has an allergy with anaphylaxis reaction and has a personal adrenaline injector with them. This includes at clinical handover in the ward or when the patient temporarily leaves the ward, such as attending scans or other tests and the personal injector is transferred with them. The patient should also be identified as a Patient of Concern (POC) during clinical handover. An allergy alert will be added to the patient's healthcare records in accordance with [SCGOPHCG Clinical Alerts Policy](#). Areas where patient journey boards (or similar) are used at the patient's bedspace, an allergy alert sign (or similar) should be placed.

Initial assessment and diagnosis

1.1 Could this be anaphylaxis?

Anaphylaxis should be considered in patients who deteriorate suddenly, especially when they display characteristic signs and symptoms of allergy, or if they have been recently exposed to a potential allergen or new medication, or if they have a known history of anaphylaxis. The presence of an allergic trigger or a history of allergy should heighten suspicion even if the patient is not in severe distress. Symptoms typically occur in two or more body systems, but this can be variable.¹

1.2 Signs and symptoms of allergic reaction¹

Reaction	Signs and symptoms include one or more of the following
Mild to moderate allergic reaction	• Rash, hives (red raised, itchy bumps), welts or flushing/erythema • Swelling of the lips, eyes or face • Itchy or tingling mouth • Stomach pain, nausea, or vomiting (in a food exposure) • In the case of sting or bites, localised swelling at sting site • Skin and mucosal changes Note: These signs and symptoms may or may not be present with anaphylaxis. Skin and mucosal changes can be subtle or absent in up to 20% of anaphylaxis cases.
Anaphylaxis	Airway • Swelling of the tongue • Difficulty swallowing or speaking • Swelling or tightness in throat • Change in voice (hoarse or croaky sounds) • Stridor (high-pitched inspiratory noise caused by upper airway obstruction) Breathing • Difficult or noisy breathing • Sudden persistent cough • Wheeze • Shortness of breath (increased respiratory rate) Circulation • Low blood pressure (hypotension) with persistent dizziness or feeling faint • Collapse/syncope • Sudden onset of pallor and floppiness (in babies and young children) • Decreased conscious level or loss of consciousness • Cardiac arrest Gastrointestinal (in a non-food exposure) • Severe nausea/vomiting/diarrhoea • Severe abdominal pain

If a patient presents with rapid development of airway, breathing and/or circulation problems, with or without skin or mucosal changes, assess the patient immediately following DRABCDE for possible anaphylaxis.¹

Anaphylaxis can be difficult to diagnose. The absence of skin signs does not rule out the diagnosis, as dermatological changes may not appear until circulation is restored. GI symptoms in a non-food exposure route may also be a sign of anaphylaxis over a mild to moderate allergic reaction. Similarly, acute deterioration and shock involves a broad differential and thus the patient's presentation should be considered in context and with early senior involvement.

In the community setting prior to administration of medication, patients and/or their carer should be educated on the clinical signs of anaphylaxis and on the appropriate course of action following any form of adverse reaction.

1.3 If symptom onset is acute

Acute onset of an illness (minutes to several hours) with simultaneous involvement of the skin or mucosal tissue, or both (e.g., urticaria, pruritus, flushing/erythema, angioedema to lips/face/eyes), and at least one of the following may suggest likelihood of anaphylaxis¹:

- Airway compromise (e.g., stridor, hoarse voice, throat tightness, tongue swelling)
- Respiratory compromise (e.g., wheeze, cough, shortness of breath). **In the event of respiratory arrest, call for assistance immediately (e.g., activate MET or follow local emergency response protocol)**
- Cardiovascular compromise (e.g., pre-syncope/syncope, hypotension). **In the event of cardiac arrest, call for assistance immediately (e.g., activate MET or follow local emergency response protocol)**
- Persistent gastrointestinal symptoms especially with non-food allergen exposure (e.g., severe abdominal pain, vomiting/diarrhoea).

Acute onset of hypotension or bronchospasm or laryngeal involvement after exposure to a known or highly probable allergen for that patient (minutes to several hours), even in the absence of typical skin involvement, could also suggest anaphylaxis.¹

If any of above two criteria is fulfilled and anaphylaxis suspected, initiate basic life support (BLS) and call for assistance (e.g., activate MET or follow local emergency response protocol) immediately. The SCGOPHCG Anaphylaxis Pathway should be activated ASAP (page 2) (refer to [section 4.0 Treatment](#)).

If anaphylaxis is not suspected, patient may be having mild to moderate allergic reaction. Monitor patients closely for two (2) hours for any signs of deterioration. Escalate care as per the Observation and Response Chart (ORC) in accordance with the [SCGOPHCG Recognising and Responding for Acute Deterioration Procedure](#).

For mild to moderate allergic reactions, consider:

- Administer PRN loratadine 10mg QID
- Administer prednisolone 25mg PO
- Once symptoms have resolved and the patient reports feeling well:
 - Chart PRN loratadine 10mg QID if not already prescribed
 - Complete [ASCIA Action Plan for Allergic Reactions \(GREEN\)](#) with the patient
 - Document care provided in the patient's healthcare record

1.4 If symptom onset is not acute

When the onset of symptoms is not acute, anaphylaxis is not likely. Consider alternative causes (e.g., syncope, panic attacks, septic shock) and treat accordingly.

Treatment

Initial treatment should not be delayed when anaphylaxis is identified, however early initiation of a MET or a community ORC escalation protocol for HITH/Home Link patients will facilitate transfer to a critical care area where patients can receive further treatment and management.

1.5 Immediate response¹:

- Remove allergen (if still present) including stopping any IV medication being infused, stay with the person, call for assistance, and locate adrenaline injector.
- Optimise the patient's position: **LIE THEM FLAT**.
 - Do not allow patient to stand or walk.
 - If unconscious or pregnant, place the patient in recovery position. If pregnant, place on left side as shown in **Figure 1**.
- If breathing is difficult, allow the patient to sit with legs outstretched.



Figure 1 Optimise patient's position³

- Give intramuscular injection (IM) adrenaline 0.5mg (0.5mL in 1mg/mL ampoule) into the lateral thigh. Alternatively, administering adrenaline via autoinjector (0.3mg dose) is also appropriate depending on equipment and medication strength available. **Adrenaline is the first line treatment for anaphylaxis.** The time of administration must be documented on the Anaphylaxis Pathway Page 2 (**Figure 2**).

ANAPHYLAXIS MANAGEMENT		
Name of doctor:		Signature:
		Date/Time:
IMMEDIATE RESPONSE	TICK ALL APPLICABLE / COMPLETED ☐ Allergen removed ☐ Optimise position (do NOT allow to stand or mobilise) ☐ LIE FLAT (default) ☐ SAT (if respiratory distress requires) ☐ IM ADRENALINE 0.5mg (lateral thigh) → ☐ IV access 18G (consider x2 large lines) → ☐ High flow OXYGEN commenced →	
RESPONSE	☐ Repeat IM ADRENALINE 0.5mg / EVERY 5 MIN as required → ☐ IV ADRENALINE infusion (if multiple IM given): <i>1mg ADRENALINE in 1L NS (or use standard infusion if immediately available). Start at 6mL/kg/hr = 0.1mcg/kg/min and increase by the starting rate every 2min if needed</i> → ☐ IV fluid 10-20mL/kg over 20-30mins (up to 50mL/kg)	
		IM ADRENALINE: (Do not include Page 1) 1 st : : 2 nd : : 3 rd : : 4 th : : 5 th : :
		O2 THERAPY ← Initial Sats: % Time: : : Flow: L/min Mode:

Figure 2 Anaphylaxis Pathway excerpt A

- For HITH/Home Link patients adrenaline is carried as part of an Anaphylaxis Equipment Kit (See Appendix A) and staff responsibilities are outlined (See Appendix B) with proper medical governance (See Appendix C). Additionally, all HITH/Home Link patients receiving IV medications will have a pre-prescribed order on the front of their medication chart for the administration of adrenaline in the event of anaphylaxis in a community setting (See Figure 2 below).

HITH ADMISSION

ONCE ONLY, PRE-MEDICATION A

e/Time scribed	Medicine (print generic name)/form	Route	Dose	Date/ or d.
	ANAPHYLAXIS			
	Epipen 300mcg Adrenaline IM			
	Adrenaline 500mcg IM			
	Adrenaline 500mcg IM			
	Adrenaline 500mcg IM			
	Adrenaline 500mcg IM			
	Normal Saline 500ml STAT			

Doctor Signature: _____

Dr Print Name: _____

Date: _____

TELEPHONE ORDER (1)

Figure 2: Example of pre-prescribed adrenaline order for HITH/Home Link patients

- Large bore IV access (e.g., 18g) should be established as soon as possible, and consider inserting two large IV lines for inpatients.
- Commence high-flow oxygen for inpatients whenever available. Document the initial oxygen saturation, time of O₂ therapy commencement, flow rate and the delivery mode (e.g., HFNP)

1.6 Inadequate response/Deterioration

- For HITH/Home Link patients call 000 and request ambulance to attend (Refer to Appendix D Algorithm). Observations should be performed every 15 minutes at a minimum until the ambulance arrives.
- Further adrenaline may be given if no response after 5 mins.
 - Repeat IM adrenaline 0.5mg every 5 mins as required and document on the Anaphylaxis Pathway page 2 (**Figure 2**).
 - If multiple IM doses are given, commence IV infusion of adrenaline (see **Table 2** for dilution). Document the time of commencement and the starting rate (**Figure 3**). Consider transfer the patient to critical care areas for advanced hemodynamic monitoring.

Dilution	Starting rate	Rate
1mg adrenaline in 1L 0.9% Sodium Chloride (NaCL) or use standard infusion if immediate available	6mL/kg/hr = 0.1microg/kg/min	Increase <i>by</i> the starting rate every 2 mins if needed.

@Table 2 Dilution - adrenaline

- Fluid resuscitation: Commence IV fluid at a rate of 10-20mL/kg over 20-30 mins (max 50mL/kg). Document time of commencement (**Figure 3**)

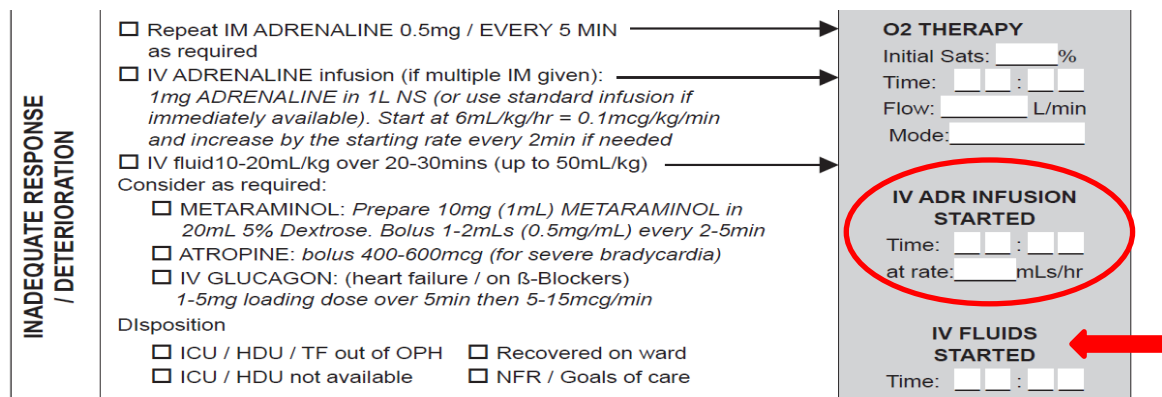


Figure 3 Anaphylaxis Pathway excerpt B

- If haemodynamically unstable, consider:
Transfer to higher level care (e.g., OPH and community setting to ED, HDU, ICU) Administer metaraminol bolus (see **Table 3** for dilution)

Dilution	Rate
10mg (1ml) metaraminol in 20mLs 5% dextrose	Bolus 1-2 mL every 2-5 mins

Table 3 Dilution - metaraminol

- If severe bradycardia, give atropine bolus 400-600microg.
- If patient has a history of heart failure and is on β-Blockers, give IV glucagon: 1-5mg loading dose over 5 mins then 5-15 microg/min.

Patient monitoring

Relapse, protracted and/or biphasic reactions may occur. True biphasic reactions are estimated to occur following 3-20% of anaphylactic reactions.

- Keep the patient in a visible bed, observe the patient for at least four (4) hours after the last dose of adrenaline following anaphylaxis. Monitor for any deterioration and document allergy (if identified) as per [SCGOPHCG Clinical Alerts Policy](#).
- Do NOT allow the patient to stand or mobilise¹⁻²:
 - For 1 hour (at minimum) post single dose of adrenaline
 - For 4 hours post the last dose of adrenaline if two (2) or more doses required
- Consider PO prednisolone 50mg if corticosteroid not already given
- Consider serial measurements of mast cell tryptase concentrations.
 - Taken during anaphylaxis, results are useful during follow-up by a Clinical Immunology or Allergy Specialist. Gold test tube is used (refer to [SCGOPHCG Venepuncture Clinical Practice Guideline Appendix 1 Evacuated Specimen Collection Guide and PathWest Test Directory for pathology collection guidelines](#))
 - Send repeat mast cell tryptase at 1-3hrs post anaphylaxis.
 - Send repeat mast cell tryptase at 12hrs post anaphylaxis or just prior to discharge whichever is later.

RED  Serum [No Gel]	Therapeutic Drug Levels, antibiotic assays and drugs of abuse N.B - a red top non-gel serum tube is preferred when collecting outside the metropolitan area. For collections in the metropolitan area please use a lithium heparin (dark green top – see below) [Use EDTA for Anti-Rejection Drugs] Lithium assays can only be performed on a serum (red top) tube
GOLD  SST / Serum [with Gel]	All Referred Biochemistry [i.e. tests that take >1day for a result] All Viral Serology Immunology All Hormones

- Re-assess the patient after four (4) hours. Consider factors such as the severity of the reaction, history of anaphylaxis, concomitant conditions, social situation when deciding if longer observation is required.
- Patients should be observed overnight if they¹:
 - Had severe or protracted anaphylaxis (e.g., required repeated adrenaline doses or IV fluid resuscitation), OR
 - Have a history of severe or protracted anaphylaxis, OR
 - Have concomitant illnesses (e.g., asthma, lower respiratory tract infection, history of arrhythmia, systemic mastocytosis), OR
 - Live alone or are remote from medical care, OR
 - Presented for medical care late in the evening OR
 - There is no adrenaline injector (EpiPen) available on discharge, or inability to fill script for an adrenaline injector.

Documentation

Document the allergen or suspected allergen in the patient's healthcare record, including food, medicine, and sting or bite exposure in the hours before anaphylaxis. For medicine allergies, include details of the adverse reaction and report via Clinical Alert (med Alert) MR002 Notification Form as per [SCGOPHCG Clinical Alerts Policy](#). Upload the anaphylaxis event and document their allergy status in the patient's My Health Record in accordance with the [NMHS My Health Record \(MHR\): System Control, Administration, Access, Upload and Security Procedure](#).

6.1 Documentation requirements for patient possession of Personal Adrenaline Autoinjector at bedside

Nursing staff must document within the nursing care plan that the patient is in possession of a personal adrenaline injector. By signing each shift, it signifies that a risk assessment has been carried out and confirms that both the patient and environment are deemed safe to keep the personal adrenaline injector on them.

However, if it is determined by either medical or nursing staff that the patient or environment are deemed unsafe to keep the personal adrenaline injector on them, this information must be clearly documented in the patient's inpatient medical records. Additionally, the presence of a personal adrenaline injector should be recorded in iCM under the 'risks' section.

If the patient self-administers their personal injector clinical staff must document on the medication chart and identify self-administration as soon as possible ([as per Medicines Management Policy and Procedure section 5.5](#)).

Discharge

Patient can be discharged four (4) hours post the last dose of adrenaline following anaphylaxis if:

- Only single dose of adrenaline was required.
- Patient is haemodynamically stable.

Discuss with the patient and their family/carer about the anaphylaxis diagnosis, the suspected allergen and the risk of biphasic reaction, and provide advice about allergy avoidance. Tailor the patient's discharge care to the suspected allergen and risk of re-exposure to ensure adequate follow-up and preventive measures.¹ In most cases this will include¹:

- EpiPen education completed and given to patient ([ASCIA Action Plan for Anaphylaxis and EpiPen Education](#))

- Advice about the allergen or suspected allergen and how to avoid it
- For patients with a medicine allergy, ASCIA provide a template for documenting the reaction ([ASCIA Record for Drug \(Medication\) Allergy](#)). This should be completed and provided to the patient to assist patient information and clinical communication.
- Advice about follow-up visits with their General Practitioner (GP) and a Clinical Immunology or Allergy Specialist. Provide the patient with copy of the discharge letter
- eReferral made to Clinical Immunology or Allergy Specialist
- [ACSQHS's Anaphylaxis Discharge Checklist and Discussion Guide](#)
- Prescribing personal adrenaline injector(s), where there is a risk of re-exposure.
 - Ensure that the patient has access to an EpiPen upon discharge. If script required, write a PBS script and obtain phone authority from the Pharmaceutical Benefits Scheme (1800 888 333). Authority approvals will be limited to a maximum quantity of 2 auto injectors at any one time
- Recognise the degree of anxiety the patient and/or their family may experience after an anaphylaxis event. Provide the patient with information about support available in the community, such as [Allergy & Anaphylaxis Australia](#) and the [ASCIA Information for Patients and Carers](#).

Provide the discharge documents, including the suspected allergen, discharge care plan and the patient's ASCIA Action Plan to their GP or ongoing clinical provider within 24 hours of discharge in accordance with the [NMHS Discharge Summary Policy](#).

Training and education

Ensure staff access available education on the use of an EpiPen®, the importance of patient positioning and consideration of repeated doses after 5 minutes, if required through the use of EpiPen® training videos.

There are e-Learning resource available from the Australasian Society of Clinical Immunology and Allergy (ASCIA).

- [ASCIA anaphylaxis e-training- Health Professionals](#)
- [ASCIA anaphylaxis e-training- Pharmacists](#)
- ASCIA anaphylaxis e-training Refresher for Health Professionals

Additionally, ensure staff administering intravenous medication for HITH/Home Link patients in the community setting can recognise and respond to anaphylactic reactions promptly and appropriately. Common triggers may include, but are not limited to penicillin, cephalosporins, amphotericin, ciprofloxacin and vancomycin.

Registered Nurses administering intravenous medications in the community setting must undergo mandatory education in the recognition and management of anaphylaxis in both the clinic setting and the community setting including the correct use of adrenaline injectors (i.e. EpiPens) on the commencement of employment. This includes an annual refresher comprising of at least one of the following, lecture, self-directed learning package, scenario-based training sessions or question and answer sessions.

Compliance, monitoring and evaluation

Compliance with this guideline will be monitored via annual auditing of documentation on the Anaphylaxis Pathway. The following indicators will be collected as part of the compliance monitoring with the [ACSQHS Acute Anaphylaxis Clinical Care Standard](#)¹:

- Proportion (%) of patients with anaphylaxis treated with IM adrenaline.

- Proportion (%) of patients treated for anaphylaxis discharged from SCGOPHCG with a completed ASCIA Action Plan for Anaphylaxis or ASCIA Action Plan for Drug (Medication) Allergy.
- Proportion (%) of patients treated for anaphylaxis who require and are supplied or prescribed adrenaline injectors post discharge from SCGOPHCG.

Related Documents

[NMHS Discharge Summary Policy](#)

[NMHS My Health Record \(MHR\): System Control, Administration, Access, Upload and Security Procedure](#)

[SCGOPHCG Venepuncture Clinical Practice Guideline](#)

[SCGOPHCG Clinical Alerts Policy](#)

[SCGOPHCG Recognising and Responding to Acute Deterioration \(RRAD\) Procedure](#)

[SCGOPHCG Anaphylaxis Pathway \(MR671\)](#)

References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). Acute Anaphylaxis Clinical Care Standard. 2021. Available from: [Acute Anaphylaxis Clinical Care Standard \(2021\) | Australian Commission on Safety and Quality in Health Care](#)
2. ASCIA. ASCIA Guideline - Acute management of anaphylaxis. 2023. Available from: [ASCIA Guidelines Acute management of anaphylaxis - Australasian Society of Clinical Immunology and Allergy \(ASCIA\)](#)
3. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards (2nd ed) Standard 8 Recognising and Responding to Acute Deterioration. Available from: [National Safety and Quality Health Service \(NSQHS\) Standards \(National Standards\) Standard 8: Recognising and Responding to Acute Deterioration](#)
4. Nursing Practice Guidelines: Intravenous Therapy. SCGOPHCG. 2004 [updated 2025]. Available from: https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/SCGH/SCGOPHCG.NPG.Intravenous_Therapy.pdf

Authorisation & Version Control

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Standards Applicable	NSQHS Standards (Version 2):  ACSQHC Acute Anaphylaxis Clinical Care Standard (2021)				

Appendix A: Anaphylaxis Kit Equipment for HITH/Home Link patients

- Laerdal pocket mask
- Adrenaline 300 mcg pre-loaded autoinjector syringe (EpiPen)
- Adrenaline 2 x 1mL of 1000 mcg/mL (1:1000) (= 4 doses of 500 mcg)
- 2 mL syringes x 4
- Drawing up needles x 4
- 23-gauge safety needles for IM injection x 4
- Sphygmomanometer
- Stethoscope
- Normal Saline 500 mL
- Pressure IV giving set
- Record of vital signs document
- Kidney dish
- Sharps box

Appendix B: Responsibilities for staff attending HITH/Home Link patients

Staff attending a community visit to administer intravenous medications must carry an anaphylaxis kit into the patient's home.

- Anaphylaxis kits are to be checked daily and restocked as required by the clinical team. The Adrenaline and Normal Saline are checked for expiration and replaced as needed.
- Adrenaline is to be kept in a protected manner inside a sealed bag with a tamper tag evident. The expiry date should be clear from the outside of the bag.
- Staff to provide patients with the following patient information:
 - On admission to HITH or non-admitted services the patient and carer receive education on the signs and symptoms of delayed allergic reactions and on the appropriate course of action following any form of adverse reaction.
 - Patients are advised to contact Home Link immediately should they experience any mild adverse reactions or alternatively present to the nearest Emergency Department as they may deteriorate over time.

Appendix C: Medical Governance for HITH/Home Link patients

- HITH patients receiving intravenous medications are under the medical governance of the HITH admitting medical team.
- Non-admitted patients receiving intravenous antibiotics are under the medical governance of an Infectious Diseases consultant in conjunction with Home Link junior medical staff. After hours, an Infectious Diseases consultant is on call. The exception to this is respiratory patients whose governance remains with the admitting respiratory team. Contact the appropriate medical team in the event of a Home Link patient having an anaphylactic reaction.

Appendix D: HITH/Home Link Anaphylaxis Algorithm

Patient develops symptoms suggestive of an allergic reaction – COULD THIS BE ANAPHYLAXIS?

NB: Typical skin (or mucosal) signs may be an early sign of further deterioration, but severe anaphylaxis may occur without typical skin (or mucosal) signs.

**ANY TYPICAL SKIN OR MUCOSAL INVOLVEMENT?
(TICK BOX / BOXES)**

☐ Urticaria	☐ Pruritis / lip tingling
☐ Angioedema lips / face / eyes	☐ Vomiting / diarrhoea / crampy abdo pain
☐ Flushing / erythema	

IMMEDIATE RESPONSE:

☐ Cease infusion
☐ Locate adrenaline autoinjector / EpiPen
☐ Monitor for symptom progression:

ANY SIGNS OF ANAPHYLAXIS? (TICK BOX / BOXES)

AIRWAY COMPROMISE	RESPIRATORY COMPROMISE	CARDIOVASCULAR COMPROMISE	PERSISTENT GI Sx IN NON-FOOD EXPOSURE (eg IV, IM, topical, contact, sting / bite)
☐ Stridor	☐ Respiratory arrest	☐ Cardiac arrest	☐ Severe abdo cramps
☐ Hoarse voice	☐ Wheeze	☐ Pre- / syncope	☐ Vomiting / diarrhoea
☐ Throat tightness	☐ Cough	☐ Hypotension	
☐ Tongue swelling	☐ Shortness of breath		

NO

YES

NOT ANAPHYLAXIS

- ☐ Symptoms settling / not progressing
- ☐ For mild / moderate allergic symptoms consider:
PO loratadine 10-20mg
- ☐ Arrange for patient to have same day medical review by Medical Team in HITH/Home Link clinic OR After hours in ED (notify team Consultant with governance on call)

ANAPHYLAXIS SUSPECTED

- ☐ In the event of Cardiac / Respiratory Arrest initiate BSL / ALS protocols
- ☐ Lie patient flat (do NOT allow to stand or mobilise)
OR sit up (if respiratory distress requires)
- ☐ **Give IM Adrenaline 0.5mg / EpiPen 0.3mg (lateral thigh)**
- ☐ **Phone ambulance – 000**
- ☐ Repeat IM adrenaline 0.5mg / EpiPen 0.3mg every 5 minutes as required
- ☐ If IV access, give 500ml N/Saline
- ☐ Transfer patient to ED via SJA + notify HITH/Home Link Consultant or if After Hours the C block CNS